

PRE-HOSPITAL CHECK CARD

IMMEDIATE EMERGENCY CARE • AIDE-MEMOIRE

INTERACTIVE — TAP ANY BOX TO TICK

Ticks and notes save with the file. Use CLEAR in the footer to reset a card. Best in Adobe Acrobat Reader.

CONTENTS

LIFE-SAVING INTERVENTIONS

- 1** Unconscious Breathing Casualty
- 2a** Adult BLS
- 2b** Under 18 (Child) BLS
- 3** Ventilation / Respiratory Support

TRAUMA

- 4** Life Threatening Bleed
- 5** Internal Bleed
- 6** Fractures
- 7** Chest Injuries
- 8** Burns / Smoke Inhalation
- 9** Spinal Injuries
- 10** Head Injuries

MEDICAL

- 11** Fits, Seizures and Convulsions
- 12** Cardiac Chest Pain
- 13** Difficulty Breathing
- 14** Allergic Reactions
- 15** Diabetes
- 16** Stroke / Unknown Illness
- 17** Sepsis – NEWS2

ENVIRONMENTAL

- 18** Hypothermia
- 19** Heat Illness
- 20** Drowning

REFERENCE

- A** Entonox
- B** ROLE
- C** Equipment Parameters
- D** Ten Second Triage (TST)
- E** S.A.M.P.L.E
- F** Monitoring / Handover
- G** Notes

L3 FREC 3 **L4** FREC 4 **EXT** extended skill – governance required

Tap any row to jump straight to that card

PRIMARY SURVEY – D R A B C

D

DANGER – scene safety

MECHANISM:

Call 999 for ANY unresponsive person

☐

TRAUMA

☐

MEDICAL

TIME

R

RESPONSE – is the casualty conscious?

NAME

AGE

PAIN / 10

ACVPU – Alert • Confusion • Voice • Pain • Unresponsive

☐

A

☐

C

☐

V

☐

P

☐

U

C

CATASTROPHIC – life-threatening bleed?

Control with:

TOURNIQUET TIME

☐

DIRECT

☐

INDIRECT

☐

TOURNIQUET

☐

PACK

A

AIRWAY

CLEAR – postural drainage / suction

SECURE – head tilt (medical) • jaw thrust (trauma)

☐

OP

☐

NP

☐

iGEL

B

BREATHING

Count for 30 seconds and double • no breaths in 10 secs = CPR

SATS %

☐

<10

☐

10 - 30

☐

>30

No breathing

insert iGel, begin CPR – Card 2A/2B

RR <10 or >30

OPA / NPA, assist ventilations – Card 3

RR 10 - 30

check neck & chest (front and back) – Card 7

TRAUMA – O₂ @15 l/min via non-rebreather maskMEDICAL – SATS <94% give O₂ via non-rebreather mask (COPD 88–92%
@2–6 l/min via nasal cannula)

PRIMARY SURVEY – C D E

C

CIRCULATION

Find pulse – adult: wrist / neck • child: arm

Check central capillary refill

Consider internal bleed – chest, abdomen, pelvis, long bones

☐ < 110 ☐ > 110

PULSE

☐ 1–2 secs ☐ +3 secs

CAP REFILL

D

DISABILITY

PEARRL • physical & sensory checks

Consider FAST test • blood glucose • temperature & BP

☐ > 4mmol/L – no action☐ < 4mmol/L – see Card 15

BGL mmol/L

TEMP °C

BP mmHg

E

EXPOSURE

Expose as appropriate • enquire SAMPLE • package as appropriate

☐ EXPOSE ☐ KEEP WARM ☐ SAMPLE ☐ PACKAGETREAT INJURIES USING THE TREATMENT CARDS
MONITOR THE PATIENT

NORMAL RATES

	RESPIRATIONS / MIN	PULSE (BPM)
ADULT	12 – 20	60 – 90
CHILD	15 – 30	70 – 140
BABY	25 – 40	100 – 150

SPO2 TARGETS

	TARGET RANGE
NORMAL	94 – 98%
C.O.P.D (2–6 l/min)	88 – 92%

UNCONSCIOUS BREATHING CASUALTY

Maintain airway using
TRAUMA – JAW THRUST
MEDICAL – HEAD TILT / CHIN LIFT

☐

Prepare suction

☐

Attempt **OP** or **NP**

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Keep warm with blanket

☐

If **BGL <4mmol/l** give Glucose

☐

TRAUMA – Treat injuries
MEDICAL – Place in recovery position

☐

Monitor and hand over to Ambulance Service

☐

NOTES

If airway cannot be maintained for a trauma patient use graduated HEAD TILT / CHIN LIFT or recovery position
Place pregnant patient on left side

ADULT BLS ALGORITHM

1 UNRESPONSIVE

Check for danger. Shout for help.

☐

2 CALL 999

Call for **any** unresponsive person – do not wait to confirm breathing. Use speakerphone.

☐

3 ASSESS BREATHING

Assess while the call connects. Agonal gasping, panting or slow laboured breathing = **cardiac arrest**.

☐

4 START CPR 30:2

100–120 compressions per minute • depth 5–6 cm
Allow full recoil. Minimise interruptions.

☐

5 ATTACH AED

As soon as one is available. Follow the AED prompts. Continue CPR while pads are applied.

☐

6 CONTINUE

Until the ambulance service takes over, the person shows clear signs of life, or you are exhausted.

☐

Sequence per Resuscitation Council UK Guidelines 2025

UNDER 18 (CHILD) BLS

1 UNRESPONSIVE

Check for danger. Shout for help.

☐

2 CALL 999

Use speakerphone. Assess breathing while the call connects.

☐

3 5 INITIAL RESCUE BREATHS

Open the airway and give **5 rescue breaths** before any compressions. Most paediatric arrests are hypoxic.

☐

4 START CPR 15:2

If trained in paediatric BLS use **15:2**. If trained in adult BLS only, use 30:2.

☐

5 COMPRESSIONS

100–120 per minute • depth **one third** of the chest
Infants: **two-thumb encircling** technique.

☐

6 ATTACH AED

AEDs may be used at **any age, including infants**. Under 25 kg use front-and-back pad placement.

☐

7 CONTINUE

Until the ambulance service takes over or the child shows clear signs of life.

☐

Sequence per Resuscitation Council UK Guidelines 2025

VENTILATION / RESPIRATORY SUPPORT

TIME CRITICAL – RESPIRATORY RATE <10 OR >30 PER MIN

Insert **OP / NP**
Prepare suction

☐

ASSIST BREATHING
GIVE OXYGEN @15 l/min

3 seconds after the patient's breath give 1 breath via BVM / Pocket Mask

☐

Continue vent support. If the patient's breaths become more than **10 seconds apart commence CPR – Card 2A / 2B**

☐

Prepare **AED** for potential use
Prepare chest, place AED next to patient, **DO NOT apply pads**

☐

LIFE THREATENING BLEED

Apply immediate **direct / indirect pressure**

☐

LIMB BLEED – Tourniquet / compression dressing
JUNCTIONAL WOUND – Pack / compression dressing

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox
GIVE OXYGEN @15 l/min via non-rebreather mask

☒

DO NOT release tourniquet
Note time of tourniquet application

☒

Arrange **URGENT** transport to hospital

☒

KEEP PATIENT WARM AND MINIMISE MOVEMENT

NOTES

Do not pull out embedded objects – pad around the object to stabilise and apply pressure with a compression dressing

INTERNAL BLEED

Visually / physically check:
CHEST / ABDOMEN / PELVIS / LIMBS

☐

Consider **Pelvic Binder**

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox
GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Arrange **URGENT** transport to hospital

☐

KEEP PATIENT WARM AND MINIMISE MOVEMENT

NOTES

Plan for the patient to deteriorate

TREATMENT OF FRACTURES

Remove watches / jewellery / clothing from the affected limb

☐

Give pain relief (check allergies)

Entonox if authorised and not contra-indicated – see Entonox card

☐

If **open fracture** pack around bone ends but **DO NOT push back in**. Apply dressing over wound

☐

Immobilise fracture using splints for lower limb injuries
Consider pelvic splint

☐

KEEP PATIENT WARM AND MINIMISE MOVEMENT

NOTES

CRUSHED LIMB

<30 minutes – release ASAP, consider tourniquet

>30 minutes and large muscle group – apply tourniquet and release limb

CHEST INJURIES

DO NOT GIVE ENTONOX

Visual / physical check of **CHEST (4 sides)**

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Seal all open wounds using chest seals
(one valved seal per lung)

☐

Pad around embedded objects – **DO NOT REMOVE**

☐

Allow patient to adopt a comfortable position, avoid laying flat
If unconscious, lay bad side up

☐

KEEP PATIENT WARM AND MINIMISE MOVEMENT

NOTES

Plan for the patient to deteriorate

BURNS / SMOKE INHALATION

Assess airway. If **soot / swelling** is present treat as **time critical** and request ambulance

☐

Remove watches / jewellery / clothes from the affected area and **cool for 20 minutes**

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox
GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Dress burns with cling film and continue to cool
DO NOT USE cling film on FACE / CHEMICAL BURNS

☐

NOTES

Plan for the patient to deteriorate

Pulse oximetry is unreliable for patients suffering smoke inhalation

DO NOT burst blisters

DO NOT remove clothing stuck to burns

Keep patient warm whilst cooling

CHEMICAL BURNS – remove contaminant and seek advice

SPINAL INJURY

Instruct patient not to move. If standing, instruct to lie down on scoop / blanket

☐

Hold head and neck in neutral alignment (**MILS**)

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox
GIVE OXYGEN @15 l/min via non-rebreather mask

☐

If moving patient, apply collar and secure to scoop / longboard

☐

NOTES

Plan for the patient to deteriorate

Plan actions for if the patient vomits

DO NOT hold head whilst the patient is standing

If unconscious and breathing, maintain MILS and refer to Card 1

HEAD INJURY

DO NOT GIVE ENTONOX

Instruct patient not to move. If standing, instruct to lie down on scoop / blanket

☐

Hold head and neck in neutral alignment (**MILS**)

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Dress wounds

☐

If moving patient, apply collar and secure to scoop / longboard
slightly head up

☐

NOTES

Plan for the patient to deteriorate

Plan actions for if the patient vomits

Once triple immobilised, slightly loosen collar

If unconscious and breathing, maintain MILS and refer to Card 1

FITS, SEIZURES AND CONVULSIONS

Clear area around patient and protect head

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

When fitting stops, assess patient
If unconscious and breathing – **CARD 1**

☐

Request ambulance if:
First fit • Repeated fitting
Fit lasting longer than 5 minutes
No responsible person present

☐

NOTES

Consider MOI – TRAUMA / MEDICAL

If patient is under 6 years old with a temperature above 38°C, remove clothing and allow to cool (febrile convulsion)

CARDIAC CHEST PAIN

Sit patient in a comfortable position, avoid lying down unless unconscious

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox, **GIVE OXYGEN** if **SATS <94%**

☐

If available, administer **1 spray of GTN** under the tongue, providing the patient has not taken Viagra or similar in <24 hours

☐

Administer **300mg ASPIRIN** if pain lasts >15 mins, providing patient is >16 years, not allergic, and has no liver disease / clotting disorder

☐

If pain persists, advise **2nd spray of GTN** at least 5 mins after the 1st dose.
DO NOT give more than 2 sprays of GTN

☐

NOTES

Plan for the patient to deteriorate
300mg ASPIRIN can be given on top of a 75mg daily dose
The patient should CHEW the aspirin

DIFFICULTY BREATHING

Remove from trigger if appropriate. Encourage patient to control breathing rate

☐

Encourage patient to adopt a comfortable position, avoid lying down unless unconscious

☐

If **SATS <94%** give **OXYGEN** @15 l/min via non-rebreather mask

☐

If known **COPD**, aim for SATS of **88–92%** @2–6 l/min via nasal cannula

☐

NOTES

Plan for the patient to deteriorate

If asthmatic, encourage the patient to use their own inhaler

ALLERGIC REACTION

Remove from trigger if appropriate. Encourage patient to control breathing rate

☐

Encourage patient to adopt a comfortable position, avoid lying down unless unconscious

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Use **ADRENALINE AUTO-INJECTOR** if available

☐

If no improvement and a 2nd pen is available, administer **5 mins after the 1st**

☐

NOTES

Plan for the patient to deteriorate
Send used injectors to hospital with the casualty

DIABETES

DO NOT GIVE INSULIN

Encourage patient to adopt a comfortable position

☐

If **BGL <4 mmol/l** and patient is **conscious** – give sugary foods / 10ml glucose gel

☐

If **BGL <4 mmol/l** and patient is **unconscious** – apply 10ml glucose gel directly onto the patient's gums with a gloved finger

☐

If unconscious and breathing – **CARD 1**

☐

If **SATS <94%** give **OXYGEN** @15 l/min via non-rebreather mask

☐

NOTES

Give up to 30ml glucose gel maximum
For high blood sugar, treat as unknown illness
MUST be seen by an ambulance crew

STROKE / UNKNOWN ILLNESS

Encourage patient to adopt a comfortable position, avoid lying down unless unconscious

☐

Consider **FAST** test for stroke

☐

EXT

Pain score >4 **GIVE ENTONOX** unless contra-indicated (see Entonox card)

☐

If not giving Entonox
GIVE OXYGEN @15 l/min via non-rebreather mask

☐

NOTES

FACE – does the patient's face droop on one side?

ARMS – can the patient raise both arms?

SPEECH – is the patient's speech slurred?

TIME – time critical

SEPSIS – NEWS2

For medical patients, consider completing NEWS2
Score each parameter, then total

PHYSIOLOGICAL PARAMETER	3	2	1	0	1	2	3
Respiration rate (per minute)	<8		9–11	12–20		21–24	>25
SpO2 Scale 1 (normal)	<91	92–93	94–95	>96			
SpO2 Scale 2 (COPD)	<83	84–85	86–87	88–92 >93 on air	93–94 on oxygen	95–96 on oxygen	>97 on oxygen
Air or oxygen?		Oxygen		Air			
Systolic BP (mmHg)	<90	91–100	101–110	111–219			>220
Pulse (per minute)	<40		41–50	51–90	91–110	111–130	>131
Consciousness (ACVPU)				Alert			CVPU
Temperature (°C)			35.1–36.0	36.1–38.0	38.1–39.0	>39.1	

NEWS2 TOTAL

Sum of all seven parameters

Complete the ePRF to generate the NEWS2 score automatically

HYPOTHERMIA

Protect from the elements and prevent further exposure

☐

Remove wet clothing, warm and insulate from heat loss, allow warm drinks

☐

Cover head if possible. Rational patients can be rewarmed by shower if available

☐

NOTES

Plan for the patient to deteriorate

Supervise rewarming

Pulse oximeter may not read

HEAT ILLNESS

Stop activity, remove to a cool location

☐

Remove excess / tight clothing

☐

Actively cool patient and monitor body temperature

☐

Rehydrate slowly (use oral rehydration fluid if available)

☐

NOTES

Plan for the patient to deteriorate

Monitor patient until handed over to ambulance and body temperature is normal

DROWNING

Rapidly remove from water and carry out primary survey

☐

If **CPR** required, give **5 initial ventilations**

☐

GIVE OXYGEN @15 l/min via non-rebreather mask

☐

Remove wet clothing, warm and insulate from heat loss

☐

DO NOT delay due to foaming – suction may be ineffective

☐

NOTES

Plan for the patient to deteriorate

Attempt CPR unless submersion time is greater than 90 mins

Vomiting is usual

Suspect hypothermia – CARD 18

ENTONOX

PAIN SCORE

0**NO PAIN****1–3****MILD****4–6****MODERATE****7–10****SEVERE****DO NOT GIVE ENTONOX****CONSIDER ENTONOX**

DO NOT GIVE ENTONOX IF:

- Pain score less than 4
- Severe head injury
- Violently disturbed psychiatric patients
- Eye surgery in the last 4 weeks
- Intestinal obstruction
- Chest injury (including penetrating torso injury)
- Diving in the last 24 hours

ROLE

RECOGNITION OF LIFE EXTINCT

Always attempt CPR unless one of the following is present

- **Valid DNAR in place**
- **Paramedic / doctor with valid ID states life extinct**
- **Decapitation**
- **Major skull / brain destruction**
- **Decomposition**
- **Rigor mortis**
- **Submersion for more than 90 mins**
- **Incineration**

EQUIPMENT PARAMETERS

AIRWAY

OP airway	Size to fit – smallest OP inserted in reverse
NP airway	12 years and over
Suction – adult	8 years and over
Suction – paediatric	Under 8 years

iGEL SIZING

Size 1	2–5 kg • 0–1 months
Size 1.5	5–12 kg • 3 months – 1 yr
Size 2	10–25 kg • 1 – 7 yrs
Size 2.5	25–35 kg • 8 – 10 yrs
Size 3	30–60 kg
Size 4	50–90 kg
Size 5	90 kg and over

VENTILATION & AED

BVM – adult	8 years and over
BVM – paediatric	Under 8 years
AED	Any age, including infants

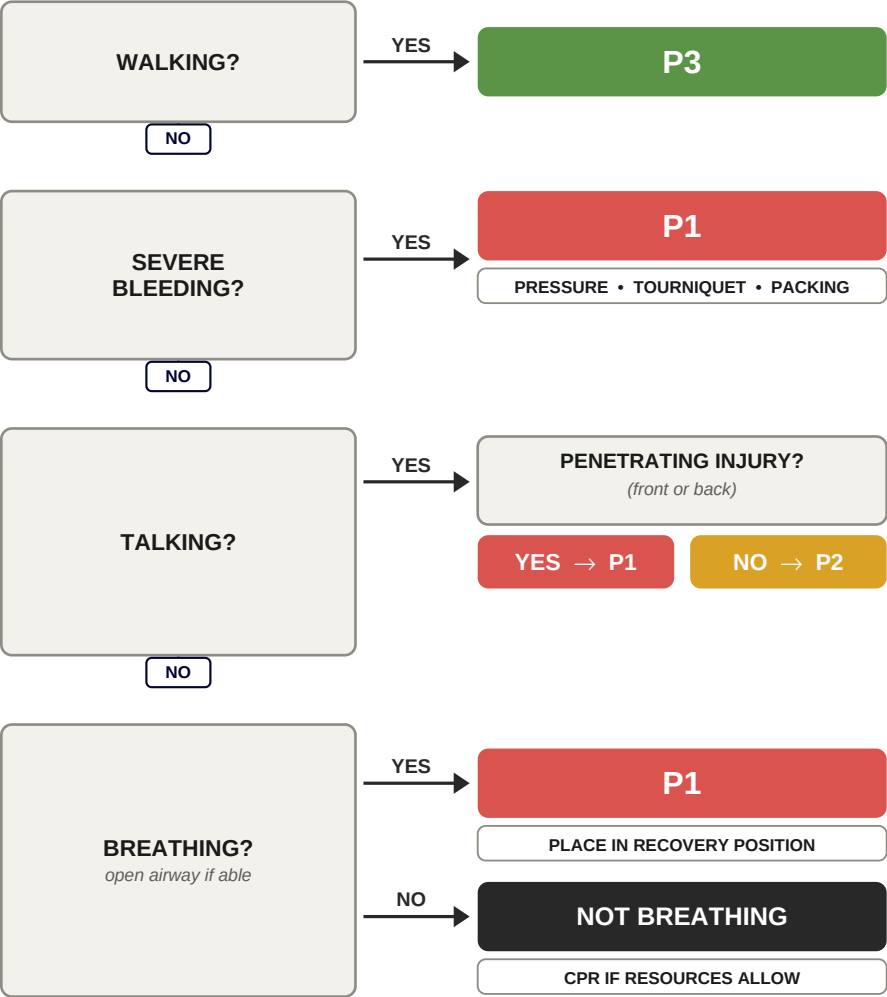
HAEMORRHAGE

Tourniquet	Size to fit
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AED PAD PLACEMENT – under 25 kg (about under 8 years): one pad mid-chest just left of the sternum, the other on the back between the shoulder blades. Pads must never touch or overlap.

TEN SECOND TRIAGE (TST)

Preliminary triage by any responder
Aim for 10 seconds per casualty



CASUALTY COUNT

P1	P2	P3	NOT BREATHING

Once clinical resources allow, undertake full physiological triage (MITT)

S.A.M.P.L.E

History taking – tap a field to record your findings

S**Signs and symptoms**

e.g. pale, sweaty, feeling sick

A**Allergies**

e.g. penicillin, nuts, latex

M**Medications prescribed**

e.g. GTN – angina, aspirin 75mg daily

P**Past relevant medical history**

e.g. heart disease, angina

L**Last oral intake**

e.g. breakfast / lunch / dinner

E**Events leading to the incident**

e.g. what happened?

MONITORING / HANDOVER

ATMIST HANDOVER

NAME

A

Age

T

Time of incident

M

Mechanism of injury

I

Injuries

S

Signs and symptoms

T

Treatment given

OBSERVATIONS

OBSERVATION	1	2	3	4
TIME				
ACVPU				
SPO2 %				
RESP RATE				
PULSE				
CAP REFILL				
BGL				
TEMP				
BP				
PAIN / 10				

NOTES